SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Docetaxel 75 mg/m² · every 3 weeks

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Source-defined breast, lung, gastric, gynaecological and castration-sensitive prostate cancer (including high-risk nonmetastatic prostate disease) · NCCP 00203 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
The aim and duration depend on the primary cancer and exact NCCP indication. Confirm the cancer-specific plan before treatment; this shared medicine schedule does not replace that discussion.

Schedule
Docetaxel 75 mg/m² by IV infusion on Day 1 every 21 days. Duration depends on the cancer and treatment aim: NSCLC has a source maximum of six cycles; other indications require an explicitly agreed course, including prostate treatment. This differs from the 100 mg/m² card.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Docetaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, mouth soreness and fatigue
- ☐ Numbness / tingling; sometimes lasting
- ☐ Fluid retention and nail changes
- ☐ Allergic infusion reaction

Discuss treatment aim, alternatives, uncertainty, fertility, contraception, access, phase changes, serious and lasting risks, supportive medicines and the urgent contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00203 v7.

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